

CHAPTER

6



Grahani

Learning Objectives

- ⇒ To learn the concept of Grahani
- ⇒ To learn about the Pratyatma Lakshana (cardinal feature) of Grahani
- ⇒ To learn the Nidana Panchaka and Samprapti of Grahani mentioned in Ayurveda literatures
- ⇒ To frame out the probable Samprapti Ghataka based on the progression of the disease
- ⇒ To learn about the Purvarupa of Grahani
- ⇒ To know about the description of Sangraha Grahani and Ghatyantra Grahani as per Madhva Nidana
- ⇒ To describe Sadhyasadyata of Grahani as per Madhavakara with the view of understanding from western physiology
- ⇒ To differentiate Grahani and Atisara
- ⇒ To know about Doshaja Atisara
- ⇒ To know the differences between Grahani Dosha and Grahani Roga

Introduction



Figure 10: Pratyatma Lakshana of Grahani

Grahani¹ is the spectrum of diseases involving *Annavaaha Srotas*. This is a *Chirakari Vikara* or long standing disease. Cardinal feature of this disease is abnormal formation of faecal matter which may range from unformed to formed stools, painful to painless defaecation, constipation to liquid stools, very less amount to large amount of stools, but the difference lies in variation in consistency of each frequency of defaecation. Profound weight loss is another major hallmark of the disease. *Grahani* is considered to be one of the *Ashtamahagada* as mentioned by *Vagbhata*. When an individual² is presented with chronic duration of unformed stools, it will lead to nutrition deficiency. There will be sequential depletion of carbohydrates, fats and proteins along with hampered absorption of vitamins, minerals.

Pratyatma Lakshana of Grahani

सा दुष्टा बहुशो भुक्तमाममेव विमुञ्चति॥२॥

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- 1 **Figure 10:** Pratyatma Lakshana of Grahani
- 2 When we talk about human body as 100%, 66% (2/3rd) of the human body is made up of water portion and rest (1/3rd) of the portion is made up of solid portion. Among the solid portion of the human body, 3/4th of the human body is made up of proteins. When an individual is having prolonged disturbances in the absorption of nutrients, there will be protein deficiency in the body. This will lead to the deficiency of the enzymes in the alimentary canal as well as enzymes of cells. There will be even deficiency of hormones too. Thus, the person who has prolonged digestive impairment, the person will have emaciation of the body along with passage of unformed stools.

पकं वा सरुजं पूति मुहुर्बद्धं मुहुर्द्वयम्।

Ma.Ni. - 4/2-3

Cardinal feature of *Grahani* is *Bahu Mala Pravritti* (large amount of liquid stools) or *Ama* (unformed stools) or *Pakva* (formed stools) or *Saruja* (painful defaecation) or *Puti* (presence of pus in the faecal matter) or *Baddha* (constipation) or *Drava Mala Pravritti* (liquid or semiliquid stools). When a person is suffering from *Grahani*, he may have passage of abnormal forms of stools in one frequency. He may not have consistent pattern of one particular character of faecal matter.

Nidana of Grahani

अभोजनादजीर्णातिभोजनाद्विषमाशनात्।

असात्म्यगुरुशीतातिरुक्षसंदुष्टभोजनात्॥4 2॥

बिरेकवमनस्नेहविभ्रमाद्व्याधिकर्षणात्।

देशकालतुवैषम्याद्वेगानां च विधारणात्॥4 3॥

Ca.Ci. - 15/42-43

- ❶ Not taking the food at all or frequent fasting³
- ❷ Excess consumption of food in a state of indigestion
- ❸ Consumption of food at improper time and amount⁴
- ❹ Consumption of incompatible food substances⁵
- ❺ Consumption of *Guru*⁶ (heavy to digest) substances
- ❻ Consumption of *Shita*⁷ (cold in touch and potency) substances
- ❼ Consumption of *Ruksha*⁸ (dry) substances
- ❽ Consumption of stale foods
- ❾ Improper *Panchakarma* treatment and improper post *Panchakarma* measures⁹
- ❿ Emaciation of the person due to other preexisting illnesses¹⁰

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- 3 When the person is doing frequent fasting, this is responsible for the appearance of hunger contractions. When the person gets hunger contractions, this will be associated with mild pain in the pit of the stomach, this is termed as hunger pangs. Hunger pangs do not begin until 12 to 24 hours after the last ingestion of food; they reach peak intensity in 3 to 4 days and gradually weaken in succeeding days. This weakening of gastric juice is leading to inactivation of pepsinogen which usually active when the pH is between 1.8 to 3.5. This is the main reason for the digestion and assimilation disturbances in the long run. Further, when the person is doing frequent fasting, it is leading to the depletion of carbohydrate, fat and protein sequentially leading to the emaciation of the body. This occurs due to the atrophy of the gastrointestinal mucosa which leads to the hampered absorption of essential nutrients.
- 4 These kinds of abnormal food patterns are responsible for retention of food substances in the stomach for more than 2 hours, sometimes 3 to 3 and ½ hours or even more. Newly consumed food will be in the centre of the stomach and previously ingested meal will move to the periphery towards the stomach wall, leading to the stretching of the stomach. Frequent exposure to these dietary patterns will lead to the infection and inflammation of the stomach, thereby there will be acidic nature formed and this will lead to the damage to the stomach wall and the person will have poor digestive juices secondary to the damage to the gastric wall.
- 5 Frequent consumption of these kinds of the substances such as consumption of milk and fish together, consumption of curds at night or consumption of curds after heating or consumption of food substances prepared out of mustard oil in South India etc, there will be inflammation of stomach. When there is inflammation in the stomach, that will lead to two of the following mechanisms:
 1. There will be increased secretion from the stomach, which will lead to the dilution of hydrochloric acid. This will lead to the dilution of gastric juice, which leads to inactivation of the pepsinogen into pepsin. This will not lead to transformation of macromolecules into intermediate or micromolecules.
 2. Depending upon the type of food intake, such as curds or fish etc, that will lead to stasis of these substances for longer time, this will dilute the gastric juice. In this situation, there will be decreased transformation of macromolecules into intermediate or micromolecules.
- 6 The substances which are heavy to digest are will take longer time to get digested in the stomach. Suppose if an individual consumes both lighter and heavier food substances, the substances which are light to digest are digested early and move to the distal portion of alimentary canal early and the substances which are heavy to digest will start moving into the distal part of gastrointestinal tract much slower pace. Thus, we can get a state of indigestion in this situation.
- 7 The consumption of *Shita* (cold) substances will lead to vasoconstriction of alimentary canal leading to decreased secretion of digestive juices right from the oral cavity to intestines. This will lead to the macromolecules which are present in the stomach may remain for longer time and thereby the person may develop infection and inflammation of alimentary canal, thus the person may develop diarrhoea.
- 8 When an individual consumes more amount of *Ruksha* (dry) substances frequently, this may stimulate excess mucus secretion from surface mucous cells of the stomach. This mucus is very viscus which coats the stomach wall of 1 mm thick and it is alkaline in nature. This will neutralise the hydrochloric acid, and this may further lead to the similar mechanism of formation of *Ama* (partially digested intermediate molecules coated over the stomach wall, development of infection and inflammation) and thus, the person may have rapid peristaltic movements leading to diarrhoea.
- 9 Improper *Panchakarma* treatment refers to either increased or decreased purification therapy. Let us take the example of *Virechana*, the administration of the medicine will lead to the inflammation of small intestine and there will be movement of pathogens present either in intracellular compartment or in interstitial compartment or in intravascular compartment will move to the alimentary canal leading to the expulsion of vitiated *Dosha* to the exterior. Improper *Virechana* therapy will lead to the abnormal or hampered movement of pathogens. Improper post *Panchakarma* procedure indicate the administration of proper *Pashchat Karma* which may disturb the functioning of *Agni* leading *Agni Dushti* for the long time. As the disease becomes chronic, the person may pass the abnormal bowel patterns thus, leading to the manifestation of *Grahani*.
- 10 When an individual is suffering from chronic debilitating illness, then such individuals will have nutrient deficiency such as carbohydrate, fat and protein loss from the body. This kind of frequency will be sequentially observed in the form of carbohydrate, fat and protein depletion. This mechanism occurs as a result of gastrointestinal mucosal atrophy. Thus, the person suffering from *Grahani*, will have decreased

- Improper measures related to one particular *Desha* such as *Jangala*, *Anupa* or *Sadharana Desha*. For example, curds intake should be avoided in *Anupa Desha* as there is every possibility of vitiation of *Kapha Dosha* which is an additional factor where *Kapha Dosha* is usually vitiated in *Anupa Desha*
- Improper measures related to one particular *Kala* such as consumption of *Shita Virya* substances in early part of night time. For example, consumption of curds in night time, as there will be vitiation of *Kapha Dosha* on that particular time¹¹
- Improper measures related to one particular season such as consumption of *Dadhi* or curds in *Vasanta Rtu*. For example, in winter season, the consumption of curds leads to the vitiation of *Kapha Dosha*
- Suppression¹²** of natural urges such as micturition, defaecation etc

Samprapti of Grahani

दुष्यत्यग्निः, स दुष्टोऽयं न तत् पचति लब्ध्वपि।

अवच्यमानं शुक्तत्वं यात्यन्नं विरूपताम्॥4॥ Ca.Ci. – 15/44

- All above etiological factors will vitiate the *Agni* (digestive fire)
- The substances which are considered to be *Laghu* (light to digest) are not going to get digested due to impaired digestive function
- This indigested or partially digested material will become *Shukrtatva* (acidic in nature) and it will be acting like toxic substance.

Samprapti Ghataka

nutrient absorption from alimentary canal and the person will have depletion of body tissues from the body tissues. The person will have depletion of *Medo Dhatu* (adipose tissue owing to conversion of adipose tissue into circulating fat in the intravascular compartment) and the person will be always lean.

- These aetiological factors are responsible for the *Viruddha* (non homologous) effect to the body. Frequent consumption of these substances will be responsible for non homogeneity to the person indulges in such kind of activities.
- Let us take the example of *Purisha* (faecal matter), *Mutra* (urine) and *Adhovata* (flatus) *Vegadharana*. When an individual suppresses these urges, that will abnormally increase the *Apanavata* in the body, which leads to the reverse movement of *Vata* in the body. This event will lead to the abnormal increase of *Samanavata* in the body. *Samanavata* is considered to be one of the six factors responsible for the proper digestion of food (*Ahara Parinamakara Bhava*). When *Samanavata* is disturbed, this may have an impact on *Ushma*, *Kleda*, *Sneha*, *Kala* and *Samayoga* of *Ahara Parinamakara Bhava*, leading to dysfunction of *Agni*. If the person still does suppression of urges, that may lead to the abnormal functioning of *Pranavata* (reversal of *Pranavata*).
- Constipation can also result from spasm of a small segment of the sigmoid colon. it should be recalled that motility even normally weak in large intestine, so that even a slight degree of spasm can often capable of causing serious constipation. After the constipation has continued for several days and excess faeces has accumulated above a spastic sigmoid colon, excessive colonic secretions often then lead to a day or so of diarrhoea. After this, the cycle begins again with repeated bouts of alternating constipation and diarrhoea.
- The presence of thirst indicates the shrinking of either intravascular compartment or interstitial compartment or intracellular compartment. This is usually attributable to the poor absorption of water from the alimentary canal. Normally, 7 to 8 litres of fluid is secreted in alimentary canal as gastro intestinal secretions and another 1.5 litres of water is consumed through oral cavity and only about 100 – 200 ml of fluid will be expelled through anus. When an individual passes more liquid to semisolid stools containing more amount of water for a longer duration may be for years may have *Grahani* manifestation.
- Laziness is attributable to the heaviness of the body parts. Heaviness can be observed in the alimentary canal also. Good digestive fire is required for the proper transformation of ingested macromolecular substances to the absorbable micromolecular substances. Macromolecules are heavier and micromolecules are lighter in nature. When an individual is having poor digestive fire, macromolecules are not converted into lighter micromolecules. That is why, the person may have laziness of the abdomen. This may lead to the transport of macromolecules and intermediate molecules into the intravascular compartment via lymphatic system. When these molecules enter the circulatory system, that will lead to the slowness of blood. As the person is having the excess macromolecules into the intravascular, interstitial and intracellular compartments, the person will present with heaviness of body parts.
- Grahani* is a chronic long-standing illness association with emaciation. Poor physical strength is attributable to poor assimilation of the nu-

- Dosha : Pitta**
- Dushya : Rasa**
- Srotas : Annavaha and Rasavaha Srotas**
- Srotodushti Prakara : Sanga and Vimargagamana**
- Agni : Jatharagnimandya and Rasa Dhatvagnimandya**
- Ama: Jatharagnimandyaajanya and Rasa Dhatvagnimandyaajanya**
- Udbhava Sthana : Amashaya**
- Sanchara Sthana: Sarva Sharira** (owing to *Rasa Dhatu* involvement in the progression of the disease)
- Vyakta Sthana : Guda**
- Rogamarga : Abhyantara and Bahya**
- Vyadhi Svabhava : Chirakari**
- Sadhyasadhya : Krichrasadhya to Asadhya** based on the duration of symptoms and severity of the illness

Purvarupa of Grahani

पूर्वरूपं तु तस्येदं तृष्णाऽऽयं बलक्षयः।

विदाहोऽस्य पाकश्च चिरात्कायस्थ गौरवम्॥4॥ Ma.Ni. – 4/4

प्रापं तस्य संदनं चिरात्पचनमम्लकः॥1१॥

प्रसेको वक्रवैरस्यमरुचिस्तुद्र लमो भ्रमः।

आनद्धोदरता छर्दिः कर्णक्ष्वेडोऽन्त्रकूजनम्॥2०॥

A.Hr.Ni. – 8/19,20

- Thirst¹³**
- Laziness¹⁴**
- Poor physical strength¹⁵**

- Partial digestion of ingested food
- Delayed digestion of ingested food¹⁶
- Heaviness of the body¹⁷

Samanya Lakshana of Grahani

सामान्यं लक्षणं काशये धूमकस्तंको ज्वरः।

मूर्च्छा शिरोरुग्विष्टम्भः श्वयथुः करपादयोः॥२१॥

A.Hr.Ni. – 8/21; A.San. – 8/23

- Emaciation¹⁸
- Burning sensation as if smoke is coming through mouth¹⁹
- Darkness in front of the eyes²⁰
- Raised body temperature²¹
- Altered state of consciousness²²
- Headache²³
- Sluggish movement of the intestines²⁴
- Swelling of the hands and feet²⁵

Types of Grahani

4 types as per Charaka, Laghu Vagbhata and Madhavakara:

- Vataja
- Pittaja
- Kaphaja
- Sannipataja

trients including glucose, sodium, potassium, calcium and water from the small and large intestines and overutilisation of essential nutrients simultaneously. Thus, the person may have poor physical strength.

- Grahani is a disease which is long-standing, emaciating illness associated with poor digestive fire. As per Ayurveda fundamentals, poor digestive fire will have other components such as Vayu, Kleda, Sneha, Kala etc. If the person is having disturbances in above said factors either one or two or more factors or all factors, that may lead to either partial digestion or delayed digestion of food.
- When an individual is presented with poor digestive fire, it is responsible for the poor nutrients absorption from the alimentary canal. This is responsible for the glucagon and epinephrine secretion from the body. Thus, the person may have transformation of glycogen to glucose for generation of energy. Later, the person will have fat mobilisation. This fat mobilisation will increase in free fatty acids in the circulation and probably this could be the reason for the heaviness of the body parts.
- Emaciation is attributable to poor assimilation of nutrients from the intestines. When you speak of nutrients, they are carbohydrates, fats and proteins and other electrolytes and water. When the individual is suffering from Grahani, he will have digestive disturbances of long duration sometimes related to years of duration. Because of poor absorption, the person will have fat depletion and protein depletion in the absence of carbohydrate depletion in the body. This will lead to the manifestation of emaciation in the patient suffering from Grahani.
- This is attributable to the formation of Shuktata (acidity) of ingested food owing to the longer stay of ingested food in the Amashaya because of abnormal faulty diet patterns such as Adhyashana or Vishamashana etc. As the food stays for a longer time in the stomach because of pyloric closure and most of the people will take carbohydrate as main diet, there will be bacterial proliferation leading to infection in the stomach. In this state, the person may develop burning sensation in the throat and mouth secondary to the infection and inflammatory reaction occurring in the stomach.
- Darkness in front of the eyes and altered state of consciousness are attributable to the syncopal episodes or hypoglycemia or hypovolemia.
- This is attributable to the infective or inflammatory mechanism occurring in the stomach.
- This is attributable to the hypoglycaemia or electrolyte imbalances manifest due to the poor essential nutrient absorption.
- When an individual is suffering from Grahani, two forms of large intestinal dysfunction may occur. They are increased peristaltic movements and decreased peristaltic movements. In case of increased peristaltic movements of large intestine, there will be passage of liquid or semi-solid stools by the patient. When the individual is having reduced peristaltic movements, increased absorption of ammonia occurs from the large intestine. Headache is said to be manifested in an individual when there is excess absorption of ammonia from the large intestine.
- This presenting complaint indicates poor digestion of ingested food. Usually, macromolecules are in unabsorbable and heavier in nature and these are not converted into lighter molecules. Thus, the person may have poor sluggish movement of molecules in the intestines.
- This is attributable to the poor protein absorption from the small intestine. This will lead to the state called hypoproteinemia in the long run and this will be responsible for the more fluid accumulation in the interstitial compartment leading to the slow pitting type of oedema in the patients suffering from this kind of illness.

Madhavakara in Madhava Nidana has mentioned 2 more variants of Sannipataja Grahani. They are Sangraha Grahani or Sangrahani and Ghatiyantra Grahani.

Vataja Grahani

कटुतिक्तकषयातिरुक्षशीतलभोजनैः।
प्रमितानशनात्यभ्यवेगनिग्रहमैथुनैः॥५९॥
करोति कुपितो मन्दमग्निरं संखाय मारुतेः।
तस्यानं पच्यते दुःखं शुक्लपार्कं खराङ्गता॥६०॥
कण्ठास्यशोषः श्रुण्णा तिमिरं कर्णयोः खनः।
पायरवणग्रीवारुजोऽभीक्ष्णं विसूचिका॥६१॥
इत्थीडा कायदौर्बल्यं वैरस्यं परिकर्तिका।
गुद्धिः सपेरसानां च मनसः सदनं तथा॥६२॥
जीर्णे जीर्यति चाध्मानं भुक्ते स्वास्थ्यमुपैति च।
स बातगुल्म होगीहाशङ्की च मानवः॥६३॥
चिरादुःखं त्रयं शुष्कं तन्वातं शब्दफेनवत्।
पुनः पुनः सृजेद्वर्चः कासश्चासादितोऽनिलात्॥६४॥

Ca.Ci. – 15/59-64

- Excess consumption of *Katu* (pungent), *Tikta* (bitter) and *Kashaya* (astringent) *Rasa* substances
- Excess consumption of dry substances
- Excess consumption of cold substances in touch and potency
- Frequent consumption of little amount of food
- Excess walking

- ✳ Frequent suppression of natural urges
- ✳ Excess sexual intercourse
- ✳ Above etiological factors will lead to the poor *Agni* (digestive fire) by abnormal *Vata Dosha*

Lakshana of Vataja Grahani

- ✳ Poor digestion of ingested food
- ✳ Partial digestion of ingested food leading to acidification of partially digested food
- ✳ Rough body parts
- ✳ Dryness of the throat and mouth
- ✳ Thirst
- ✳ Appetite
- ✳ Blurring of vision
- ✳ Unnatural sounds in the ears (tinnitus)
- ✳ Pain in the sides of the chest, thighs, groins, nape of neck
- ✳ *Visuchika* (upward and downward movement of vitiated *Dosha*)
- ✳ Discomfort in the chest
- ✳ Emaciation
- ✳ Physical weakness
- ✳ Abnormal taste perception
- ✳ Cutting type of pain in the perianal region
- ✳ The person suffering from *Grahani* will have desire to take food substances having six *Rasa*
- ✳ Sluggish mental activities
- ✳ Distension of the abdomen whether the person takes the food or not
- ✳ After the consumption of food, the person will have subsidence from the symptoms
- ✳ Differential diagnosis for *Vataja Grahani* is *Vataja Gulma*, *Hridroga* and *Pliha roga*.
- ✳ The person will pass the bowels with the characters of liquid consistency, dry in nature, delay in the process of defaecation, presence of *Ama* in the faecal matter, thin faecal matter
- ✳ Frequent passage of loose stools
- ✳ Associated with cough, breathlessness

Pittaja Grahani

कडुजी विदाहाम्लक्षारायैः पित्तमुल्बणम्।
अग्निमालावयन्ति जलं तप्तमिवानलम्॥६५॥
सोऽजीर्णं नीलपीताभं पीताभः सार्यते द्रवम्।
नृत्यम्लोद्गारहृत्कण्ठदाहारुचिर्दृढितः॥६६॥

Ca.Ci. – 15/65, 66

- ✳ Excess consumption of *Katu* (pungent) and *Amla* (sour) *Rasa* substances

- ✳ Consumption of food in the state of indigestion
- ✳ Excess consumption of *Vidahi* (producing burning sensation and thirst) substances
- ✳ Excess consumption of *Kshara* (alkaline) substances
- ✳ Due to the above etiological factors, the person will have poor digestive function

Lakshana of Pittaja Grahani

- ✳ Bluish or yellowish or yellow discolouration of liquid to semiliquid stools
- ✳ Presence of pus in the liquid to semiliquid stools
- ✳ Sour eructations
- ✳ Burning sensation in the chest and throat regions
- ✳ Tastelessness
- ✳ Thirst

Kaphaja Grahani

गुर्वतिस्निग्धशीतादिभोजनादतिभोजनात्।
भुक्तमात्रस्य च स्वप्नाद्धन्त्यग्निं कुपितः कफः॥६७॥
तस्यात्र पच्यते दुःखं हल्लासच्छर्धरोचकाः।
आस्योपदेहमाधुर्यकासष्ठीवनपीनसाः॥६८॥
हृदयं मन्यते स्त्यानमुदरं स्तिमितं गुरु।
दुष्टो मधुर उद्गारः सदनं स्त्रीष्वहर्षणम्॥६९॥
भिन्नामश्लेष्मसंसृष्टगुरुवर्चः प्रवर्तनम्।
अकृशस्यापि दीर्घत्वमालस्यं च कफात्मके॥७०॥

Ca.Ci. – 15/66-70

- ✳ Excess consumption of *Guru* (heavy to digest) substances
- ✳ Excess consumption of unctuous substances
- ✳ Excess consumption of cold substances
- ✳ Excess consumption of food substances in excess quantity
- ✳ *Kapha Dosha* gets vitiated by above said etiological factors and soon after the consumption of food

Lakshana of Kaphaja Grahani

- ✳ Digestion of food with difficulty
- ✳ Nausea
- ✳ Vomiting
- ✳ Tastelessness
- ✳ Coating over the tongue
- ✳ Sweetish taste in the mouth
- ✳ Cough with sputum
- ✳ Running nose
- ✳ Heaviness of the chest
- ✳ Sensation of the covering of the body with wet cloth

- Heaviness of the body parts
- Sweet taste eructations
- Sluggish movement of body parts
- Aversion towards sexual intercourse
- Unformed stools with the presence of mucus
- Passage of faecal matter which is heavy which will sink in the water
- Considerable amount of physical debility even though the patient is not emaciated
- Laziness

Sangrahani

(अन्नकूजनमालस्यं दौर्बल्यं सदनं तथा।
द्रवं शीतं घनं स्निग्धं सकटीवेदनं शकृत्॥1॥
आमं बहु सपैच्छिल्यं सशब्दं मन्दवेदनम्।
पक्षान्मासाद्दृशाहाद्वा नित्यं वाऽप्यथ मुञ्चति॥2॥
दिवा प्रकोपो भवति रात्रौ शान्तिं ब्रजेच्च सा।
दुर्विज्ञेया दुश्चिकित्स्या चिरकालानुबन्धिनी॥3॥
सा भवेदामवातेन संग्रहग्रहणी मता।

Ma.Ni. - 4/17-1-4

This is the advanced stage of *Sannipataja Grahani*. Following *Lakshana* are observed in the patient of *Sangrahani*.

- Gurgling sound in the abdomen
- Laziness
- Weakness
- Sluggish movement of the body parts
- Pain in the waist region
- Faecal matter associated with liquid to semiliquid stools, cold in touch, heavy, unctuous, unformed, large quantity, slimy character with passage of flatus with sound and mild pain abdomen
- Initially, the worsening of the symptoms occurs in one month. Later the symptom free period reduces 15 days and later to 10 days.
- In severe case, the worsening of the symptoms occurs daily. Peculiarity of the illness is worsening of the symptoms is seen in day time and in night time, the person will not have any symptoms.
- This disease is a chronic lingering disease and it is very difficult to diagnose and to have appropriate management is also very difficult.
- The person will have features of *Saama Vata* during the progression of the illness.

Ghatyantra Grahani

स्वपतः पार्श्वयोः शूलं गलज्जलघटीध्वनिः।
तं वदन्ति घटीयन्त्रमसाध्यं ग्रहणीगदम्॥4॥

Ma.Ni. - 4/17-4

- In this type of *Grahani*, the person will have presenting complaint of pain in the flanks in lying position
- There will be production of sound similar to the pot with some amount of water inside.
- This is called as *Asadhya* (incurable).

Samatva and Niramatva in Grahani²⁶

दोषं सामं निरामं च विद्यादत्रातिसारवत्॥1॥8॥
संसृष्टमेभिर्दोषैस्तु न्यस्तमप्यवसीदति।
पुरीषं भृशदुर्गन्धि पिच्छिलं चामसंज्ञितम्॥1॥2॥

Ma.Ni. - 4/18

एतान्येव तु लिङ्गानि विपरीतानि यस्य वै।
लाघवं च विशेषेण तस्य पक्कं विनिर्दिशेत्॥1॥3॥

Ma.Ni. - 3/12,13

- *Samatva* and *Niramatva* of *Grahani* should be considered similar to the *Lakshana* observed in *Saama* and *Nirama* stage of *Atisara*.

Sama Atisara Lakshana

- When the vitiated *Dosha* mixed with faecal matter associated with *Ama* will have following features.
- The faecal matter will sink in water, indicates the *Gurutva* of the faecal matter
- Very foul smelling
- Slimy nature

Nirama Atisara Lakshana

- The faecal matter will float in the water.
- The faecal matter will not have foul smell and non slimy in nature.

Asadhya Lakshana of Grahani²⁷

लिङ्गैरसाध्यो ग्रहणीविकारो यैस्तैरतीसारगदो न सिध्येत्।
वृद्धस्य नूनं ग्रहणीविकारो हत्वा तनुं नैव निवर्तते च॥1॥9॥
(बालके ग्रहणी साध्या यूनि कृच्छ्रा समीरिता।
वृद्धे त्वसाध्या विज्ञेया मतं धन्वन्तरेरिदम्॥1॥१॥)

Ma.Ni. - 4/19

- The *Asadhya Lakshana* of *Atisara* should be considered in analysing the *Asadhyata* (incurability) of *Grahani* illness also.
- In elderly individuals, this disease is definitely incurable²⁸.

G
K

26 *Samatva* and *Niramatva* of *Grahani* can also be similar to the clinical features of *Samatva* and *Niramatva* of *Atisara*. The relevance of these *Lakshana* we have to see in the context of *Atisara* only. Due to the voluminous content of the textbook, the descriptions are not elaborated in this chapter.

27 *Asadhya Lakshana* (incurability) of *Grahani* are similar to the *Lakshana* mentioned in the context of *Atisara*. Therefore, relevance of the *Lakshana* we have to look in the context of *Atisara* only.

28 First of all, there will be extreme tissue depletion (*Dhatu Kshaya*) secondary to old age is observed as a natural phenomena. When one

❖ In children, *Grahani* is *Sadhya* (curable)²⁹

❖ In middle age, *Grahani* disease is *Krichrasadhya* (curable with difficulty)

Difference between Atisara and Grahani: Both diseases are considered to be responsible for affliction of *Mahasrotas*. There are certain factors which differentiate the two conditions.

Atisara	Grahani
Atisara primarily involves Purishavaha Srotas	Grahani primarily involves Annavaha Srotas
Short duration of the illness is the hallmark of this disease	Long duration of the illness is the hallmark of this disease
Pratyatma Lakshana of Atisara is Gudena Atidrava Mala Pravritti (increased frequency of liquid to semiliquid stools)	Pratyatma Lakshana of Grahani is Pakva (formed stools) or Ama (unformed stools), Bahu (in large quantity) or Saruja (painful defaecation) or Puti (defaecation with the presence of pus) or Baddha (constipation) or Drava (liquid to semiliquid) Mala Pravritti
Profound weight loss is not usually seen in the patient of Atisara	Profound weight loss is usually in the patient of Grahani
Atisara is Ashukari Vikara	Grahani is Chirakari Vikara
Atisara usually have good prognosis	Grahani usually have poor prognosis

Difference between Grahani Dosha and Grahani Roga

Grahani Dosha	Grahani Roga
This is a state prior to Dosha Dushya Sammurchana	This is a state after the Dosha Dushya Sammurchana
In this, only abnormal functioning of Agni is seen	In this, not only abnormal functionin of Agni is seen, even the features of Rasa Kshaya etc are also seen
Vishamagni, Mandagni and Tikshnagni are the types of Grahani Dosha as per Charaka	Vataja, Pittaja, Kaphaja, Sannipataja are the types of Grahani as per Charaka
The duration of Grahani Dosha is usually shorter	The duration of the Grahani Dosha is chronic
Based on the concept of cause and effect, Grahani Dosha can lead to the manifestation of Grahani Roga	Grahani Roga may not lead to Grahani Dosha, as Grahani Roga is the effect and Grahani Dosha is a cause

Key points of the chapter

सा दुष्टा बहुशो भुक्तमाममेव विमुञ्चति॥२॥ पक्वं वा सरुजं पूति मुहुर्बद्धं मुहुर्द्रवम्।

Ma.Ni. – 4/2-3

❖ In the individual who is suffering from *Grahani* may sometimes pass liquid stools, sometimes he may pass hard stools, sometimes he may pass the stools with pain, sometimes he may pass the stools mixed with pus, sometimes he may pass liquid stools.

❖ This is a spectrum of diseases afflicting *Annavaha Srotas*, later afflicting *Purishavaha Srotas* and *Rasavaha Srotas* which is a chronic disease associated with profound weight loss and abnormal consistency of faecal matter. In this disease, metabolic disturbances are observed.

पूर्वरूपं तु तस्येदं तृष्णाऽऽलस्यं बलक्षयः। विदाहोऽन्नस्य पाकश्च चिरात्कायस्य गौरवम्॥४॥

Ma.Ni. – 4/4

❖ *Purvarupa* observed during the progression of *Grahani* are thirst, laziness, loss of strength, improper and delayed digestion of food.

सामान्यं लक्षणं काश्ये धूमकस्तमको ज्वरः। मूर्च्छा शिरोरुग्विष्टम्भः श्वयथुः करपादयोः॥२॥१॥

A.Hr.Ni. – 8/21; A.San. – 8/23

❖ Emaciation, burning sensation as if the smoke is coming through mouth, darkness in front of the eyes, raised body temperature, altered state of consciousness, headache, reduced peristaltic movements, swelling of the hands and feet are the common general features observed in the patient of *Grahani*.

❖ *Sankhya Samprapti* of *Grahani* are 4 types as per *Charaka*: *Vataja*, *Pittaja*, *Kaphaja* and *Sannipataja*; where as *Madhava Nidana* has added two more peculiar types of *Grahani* viz. *Sangrahani* (*Sangraha Grahani*) and *Ghatiantra Grahani*.

❖ Nature of *Purisha* in the patient of *Grahani* is similar to that of the features of *Purisha* of *Atisara*.

makes the diagnosis of *Grahani*, two factors are considered – duration of the illness and cardinal feature of the disease. In other words, the elderly individual suffering from *Grahani* will have even extreme body tissue depletion. Hence, it is incurable in old age.

29 In children, there will be improperly formed body tissues. *Grahani* is a chronic disease. The children will have easy adaptation to the lifestyle changes which may be suggested by the clinician and they respond well to the management. Hence, this is easily curable in children.

Probable questions asked in the university examinations

Long essay questions

- ⊗ Define Grahani, explain Nidana Panchaka of Grahani
- ⊗ Explain Grahani in detail
- ⊗ Write Nidana, Purvarupa, Rupa, Pratyatma Lakshana, Bheda and Sadhyasadyata of Grahani

Short essay questions

- ⊗ Explain Sangraha Grahani
- ⊗ Explain Samanya Lakshana of Grahani
- ⊗ Explain Sama and Nirama Purisha Lakshana of Grahani
- ⊗ Describe Vataja Grahani
- ⊗ Explain Samprapti and Samprapti Ghataka of Grahani
- ⊗ Explain Asadhya Lakshana of Grahani

Short answers

- ⊗ Define Grahani
- ⊗ Write the difference between Grahani Roga and Grahani Dosha
- ⊗ Enumerate the types of Grahani as per Madhavakara
- ⊗ Write the Pratyatma Lakshana of Grahani

MCQs

1. Ghatyantra Grahani is the term described by

- a. Charaka; b. Sushruta;
c. Madhavakara; d. Vagbhata

Answer: (c)

2. Types of Grahani mentioned by Charaka are

- a. 4; b. 5;
c. 6; d. 3

Answer: (a)

3. Which of the following is false in case of Grahani?

- a. Ashukari Vikara;
b. Chirakari Vikara;
c. Abnormal and unstable consistency of faecal matter;
d. Annavaha Srotas affliction

Answer: (a)

4. Srotas afflicted during the progression of Grahani is/are

- a. Annavaha Srotas;
b. Purishavaha Srotas;
c. Rasavaha Srotas;
d. All of the above

Answer: (d)

5. Which of the following type of Grahani is having the features of Saama Vata?

- a. Kaphaja Grahani;
b. Sangrahani;
c. Pittaja Grahani;
d. Ghatyantra Grahani

Answer: (b)

6. Desire to take all six Rasa substances is seen in which type of Grahani?

- a. Vataja; b. Pittaja;
c. Kaphaja; d. Sannipataja

Answer: (a)

7. Which of the following type of Grahani mimics the features of Amavata?

- a. Sannipataja; b. Vataja;
c. Pittaja; d. Kaphaja

Answer: (b)

8. The symptoms of Sangraha Grahani is worsened in which of the following time period?

- a. Day time; b. Night time;
c. Early morning; d. Late night

Answer: (a)

9. Streeshu Aharshana (aversion towards women or sexual intercourse) is observed in which of the following type of Grahani?

- a. Vataja Grahani;
b. Kaphaja Grahani;
c. Vataja Jvara;
d. Vataja Arshas

Answer: (b)

10. Which of the following condition can be equated with Grahani Roga?

- a. Diarrhoea;
b. Abdominal swelling;
c. Sprue;
d. Haemorrhoids

Answer: (c)

Chapter resources

- ⊗ Charaka Samhita, Chikitsa Sthana 15th chapter - Grahani Dosha Chikitsa Adhyaya
- ⊗ Madhava Nidana 4th chapter - Grahani Nidana Adhyaya
- ⊗ Ashtanga Hridaya Nidana Sthana 8th chapter - Atisara Grahanidosha Nidana Adhyaya
- ⊗ Ashtanga Sangraha Nidana Sthana 8th chapter - Atisara Grahanidosha Nidana Adhyaya

- ⊛ Golwalla's medicine for students by Aspi F Golwalla and Sharukh A Golwalla, edited by Milind Y Nadkar, 25th edition, Jaypee Brothers Medical Publishers (P) Ltd, New Delhi, 2017
- ⊛ The Merck Manual of Diagnosis and Therapy by Robert S Porter and Justin L Kaplan, 19th edition, Merck Sharp & Dohme Corp, New Jersey, 2011
- ⊛ Chamberlain's symptoms and signs in clinical medicine by Colin Ogilvie and Christopher C Evans, 12th edition, Butterworth-Heinemann Publishers, 1997
- ⊛ Best & Taylor's physiological basis of medical practice by O P Tandon and Y Tripathi, 13th edition, Wolters Kluwer Health (India) Pvt. Ltd, Gurgaon, 2012
- ⊛ Textbook of Medical physiology by Arthur C Guyton and John E Hall, 11th edition, Saunders Elsevier publications, Philadelphia, 2006
